SAMPLE ANSWER · DISTAL RADIUS FRACTURE · CASE 2

Sample 7-minute ISBAR: Liam

Liam O'Connor · 23-year-old man · Outpatient hand therapy clinic, regional public hospital · about 1037 spoken words (≈ 7.4 min at 140 words per minute)

I Introduction

66 words

"Hi, I'm the occupational therapy student who has been reviewing Liam O'Connor in hand therapy today. Liam is a 23-year-old apprentice electrician whose right distal radius fracture was fixed with K-wires three weeks ago.

"I need to discuss him urgently, because his pin sites show signs of possible infection, he felt hot and shivery last night, and he's about to go back to work this afternoon."

S Situation

143 words

"I was asked to review Liam's splint and check his pin sites. When I removed the splint, there was a sour smell and the padding around the wires was stained yellow-green. Two of the three pin sites are red and puffy, with redness spreading about two centimetres from each wire, and there's thick, creamy discharge crusted around one. The back of his hand is swollen and warmer than his left. He reports three days of increased throbbing and says he felt 'hot and shivery' last night.

"He's also told me he's been taking the splint off at work to pull cable through ceilings and feed conduit.

"Right now, his site supervisor is on the phone asking whether Liam can come straight back to finish a switchboard job, and Liam is pulling on his hoodie to leave. He's cheerful and says he'll 'be right'."

B Background

166 words

"Liam is right-handed. He fell from a ladder at work three weeks ago, and his fracture was managed with closed reduction and percutaneous K-wire fixation. The wire ends protrude through the skin on the back of his wrist and are protected by a thermoplastic forearm splint. At his review two weeks ago, the pin sites were documented as clean and dry, with good finger range, and the latest orthopaedic letter describes the X-ray position as 'satisfactory'.

"He has an accepted WorkCover Queensland claim and lives with his girlfriend. He smokes about 15 cigarettes a day. The orthopaedic registrar gave him a prescription at his last visit, which he didn't fill.

"There are several gaps I'd want to clarify: what that prescription was for, whether anyone has looked at the pin sites in the last two weeks, what pin-site care he was taught, when the wires are due out and what he's allowed to lift. I also don't know whether his employer has received any written restrictions."

A Assessment

293 words

"I'm concerned that Liam has signs of a pin-site infection that may be spreading. Spreading redness, swelling, purulent discharge, odour, increasing pain and warmth together are more

than mild irritation around wires. Feeling hot and shivery overnight suggests he may be developing a systemic response, so he needs medical assessment today. Untreated infection around wires can track deeper towards the bone, and it may affect whether the wires can stay in and whether the fracture position holds.

“Several factors may have contributed. Taking the splint off and pulling cable puts movement and load through the wires and exposes them to dust and dirt. Smoking impairs wound and bone healing. The unfilled prescription means that a treatment the team intended may not have happened. Heavy pulling three weeks after fixation also risks loss of fracture position, and I can’t see a recent X-ray.

“Liam’s cheerfulness and ‘barely notice it’ could easily reassure us falsely. The employer’s calls and the WorkCover form also create pressure to give a quick capacity answer, but I can’t give any update on his capacity until the infection has been reviewed and the surgeon has confirmed his restrictions. His requests for a surfing cover and gym advice aren’t today’s priority either. Getting open pin sites wet, especially in the sea, is a concern, and that’s for the surgeon to advise on.

“Occupationally, his work involves gripping, pulling, using tools and working at heights, which are currently unsafe for his right hand. His income, apprenticeship progress and social life clearly matter to him, and acknowledging that will help keep him engaged.

“As a student, I won’t diagnose an infection or decide on antibiotics, but I think this warrants same-day orthopaedic or medical review, and he shouldn’t return to site this afternoon.”

R Recommendation

369 words

“First, I’d cover the pin sites with a clean, dry dressing according to the clinic’s protocol, support the wrist without putting the soiled padding back, and ask Liam to stay so he can be reviewed. I’d explain in plain terms that this could become a serious infection that threatens the wires and his recovery, and I’d acknowledge that tonight is his girlfriend’s birthday.

“Second, I’d like your help to contact the orthopaedic registrar or fracture clinic now to request a same-day review. If they can’t see him promptly, the usual pathway would be the emergency department, especially given his fever symptoms. I’d ask the team to check his observations and pin sites, consider an X-ray, decide on treatment and clarify what the unfilled prescription was for.

“For the MDT: nursing could take his temperature and observations while he waits and reinforce pin-site care. Orthopaedics would need to give clear, written restrictions on splint wear, lifting and work, and the plan for wire removal. Pharmacy or his GP could help with any medication and with smoking cessation support, because quitting supports bone and wound healing, and Quitline is available on 13 QUIT.

“With his employer, I’d only share, with Liam’s consent, that he’s having a medical review today and shouldn’t return to site until written restrictions are provided. Work capacity should come through the treating doctor’s WorkCover Queensland certificate and his case manager, not a verbal update from me.

“For OT assessment, once he’s been reviewed, my principal short-term assessment would be observed performance of his essential daily and work-related tasks within his restrictions, such as managing his splint himself, dressing, and one-handed tool use, noting technique, pain and whether he keeps the splint on. Longer term, I’d use the COPM to establish what matters most

to him, likely electrical work, surfing and the gym, and use it to guide a graded return-to-work plan with WorkCover.

"Intervention would include re-padding or replacing the splint, pin-site care education with teach-back, problem-solving how to protect the wrist at work, and a written suitable-duties summary once the surgeon sets restrictions. I'd follow up after the medical review. For now, the priority is that Liam is seen today rather than going back to site."

Coaching notes *not spoken*

The immediate priority

Spreading redness, pus, odour, increasing pain and feeling hot and shivery around percutaneous wires need same-day orthopaedic or medical review, and Liam should not return to site until reviewed and restricted in writing.

Distractors in this case

- **Employer's calls and the WorkCover form** — capacity cannot be stated until the infection and restrictions are clarified; certification goes through the treating doctor.
- **"Barely notice it" and his cheerful manner** — self-report conflicts with objective signs.
- **Surfing cover, gym and the birthday dinner** — real goals, but not today's decision; water exposure with open pin sites is a surgeon's call.
- **"Satisfactory" X-ray in the latest orthopaedic letter** — may predate heavy use without the splint.

Missing information to clarify

- What the unfilled prescription was for.
- Pin-site care taught and any review since two weeks ago.
- Planned wire removal date and lifting restrictions.
- Whether written restrictions were given to the employer.

Useful references

- Australian Commission on Safety and Quality in Health Care, NSQHS Standards: Preventing and Controlling Infections Standard and Recognising and Responding to Acute Deterioration Standard.
- WorkCover Queensland, information on work capacity certificates and suitable duties.

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